



Private Equity in Health Care

American Board of Medical Specialties

September 16, 2026



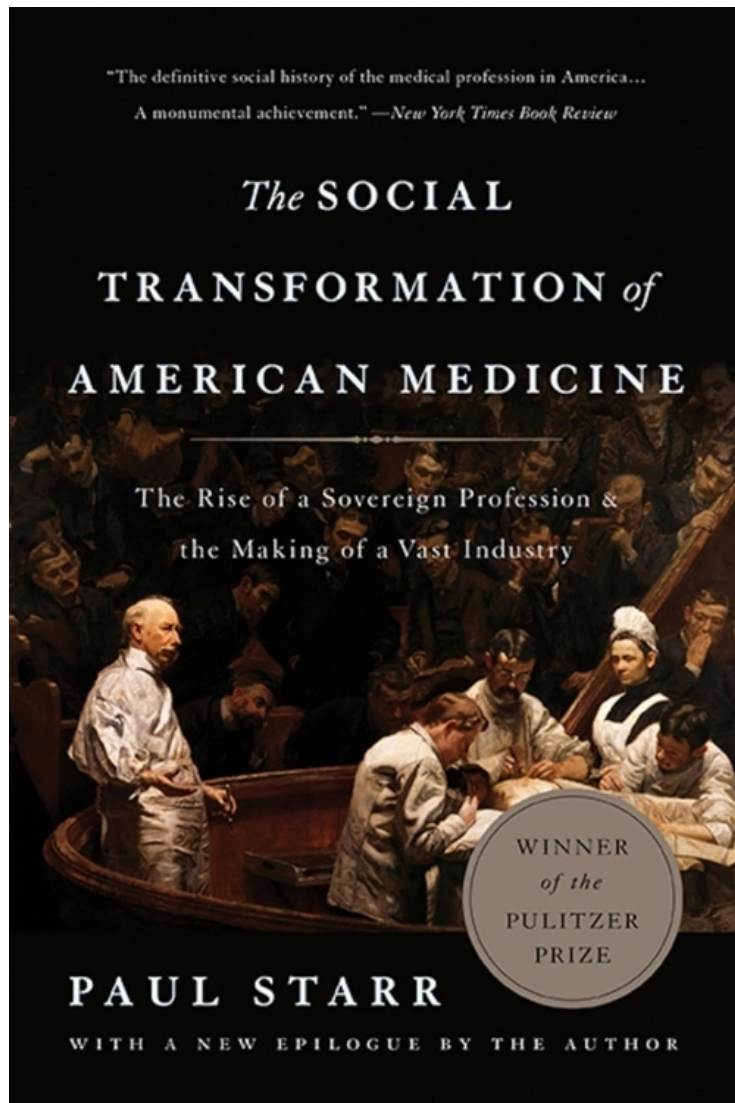
Zirui Song, MD, PhD
Harvard Medical School
Massachusetts General Hospital



1982



2024



The NEW ENGLAND JOURNAL of MEDICINE

MEDICINE AND SOCIETY

Debra Malina, Ph.D., *Editor*

The Financialization of Health in the United States

Joseph Dov Bruch, Ph.D., Victor Roy, M.D., Ph.D., and Colleen M. Grogan, Ph.D.

From early fears of a burgeoning medical-industrial complex¹ to more recent critiques of corporate greed in medicine,² observers have long decried the profit motives embedded in the U.S. health care system. While much of the focus has been on corporate influences in health care, a critical dynamic has remained largely obscured: financialization. As defined by social scientists and historians, financialization refers to the growing influence of financial markets, motives, institutions, and elites in our economy and society.³ This dynamic encompasses the expanding influence of financial actors — including commercial and investment banks, private equity (PE) firms, venture capital firms, and other types of investors — and a shift in the business of non-finance-related entities away from trade and commodity production toward new financial channels and maneuvers.⁴

We believe that health policy discussions con-

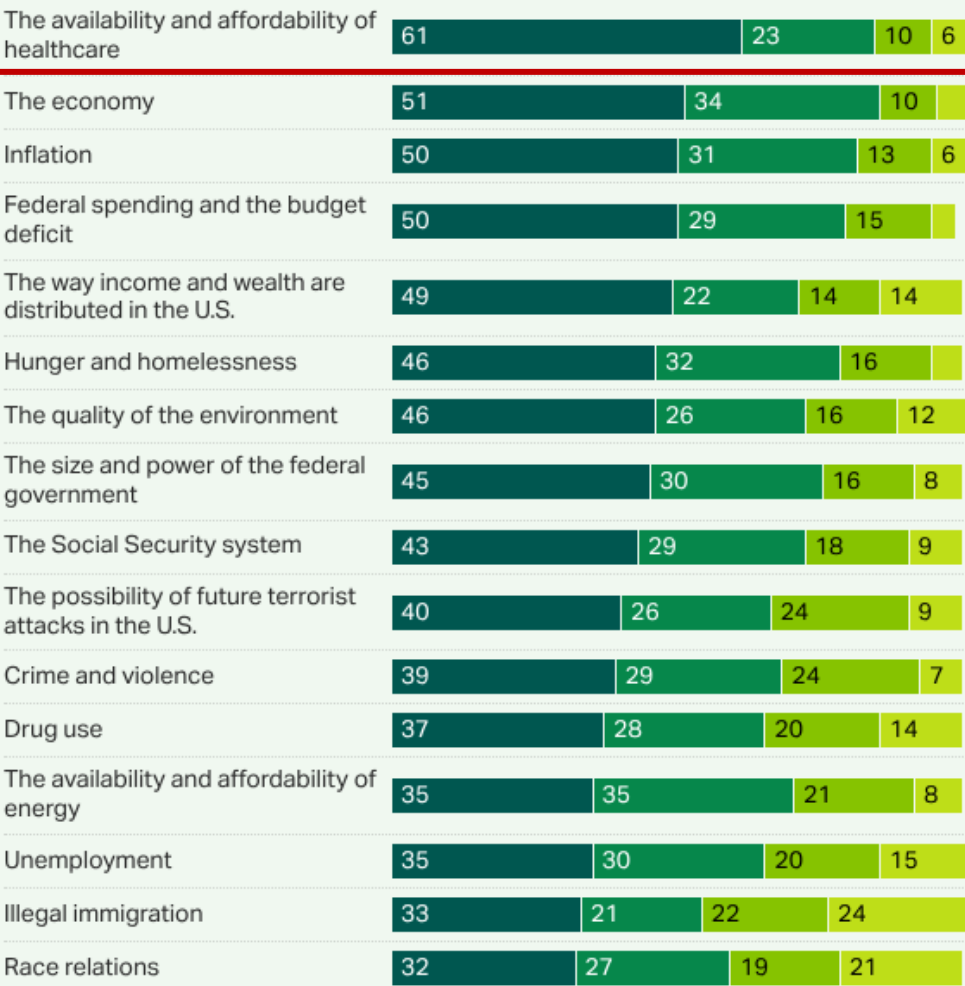
“Privatization,” on the other hand, refers to the ownership status of firms and describes the growing role of private-sector actors in the provision of health care resources, such as the rising prominence of private insurance companies in the Medicare program under Medicare Advantage.⁷

“Financialization” builds on, but also departs from, these previous and better-studied processes affecting health care. It involves the transformation of public, private, and corporate health care entities into salable and tradable assets from which the financial sector may accumulate capital.⁸ Financialization captures the new forms of financial-sector ownership and control in the U.S. health care system, as well as the demands of financial markets for short-term profit growth and the distribution of this growth to financial actors that are external to health care entities and U.S. households.

Americans' Worries About Key Issues, 2026

Next, I'm going to read a list of problems facing the country. For each one, please tell me if you personally worry about this problem a great deal, a fair amount, only a little or not at all?

% Great deal % Fair amount % Only a little % Not at all

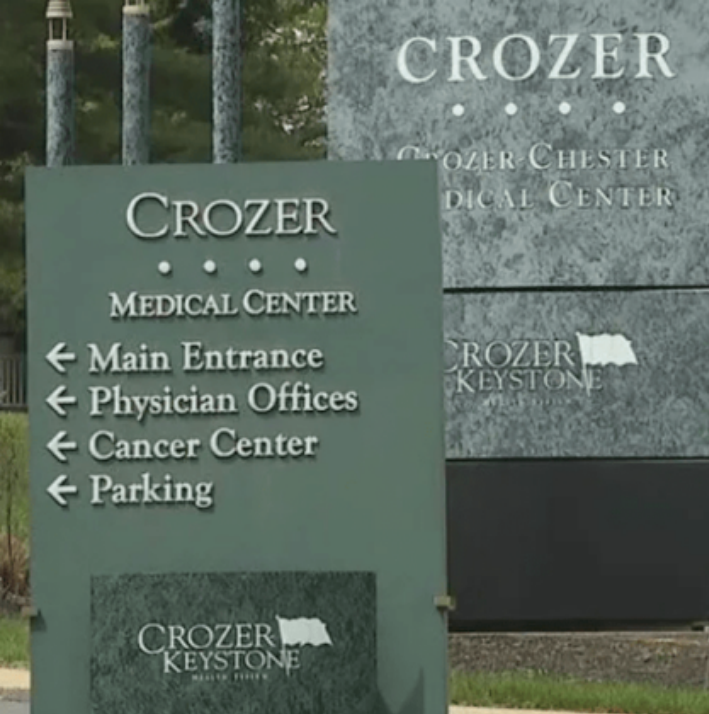


March 2-18, 2026
Those with no opinion are not shown.

Recent Trend in Americans' Worry About Key Issues

% Worry a great deal

	2024	2025	2026	Change, 2025-2026
	%	%	%	pct. pts.
Social Security system	43	52	43	-9
The economy	52	60	51	-9
Crime and violence	53	47	39	-8
Illegal immigration	48	40	33	-7
Inflation	55	56	50	-6
Hunger/Homelessness	52	50	46	-4
Federal gov't size/power	--	48	45	-3
Federal spending/budget deficit	51	53	50	-3
Drug use	45	39	37	-2
Race relations	35	33	32	-1
Unemployment	33	36	35	-1
Energy	37	35	35	0
Terrorism	43	40	40	0
Income/Wealth distribution	--	48	49	1
Healthcare	51	59	61	2
The environment	37	43	46	3



BREAKING NEWS

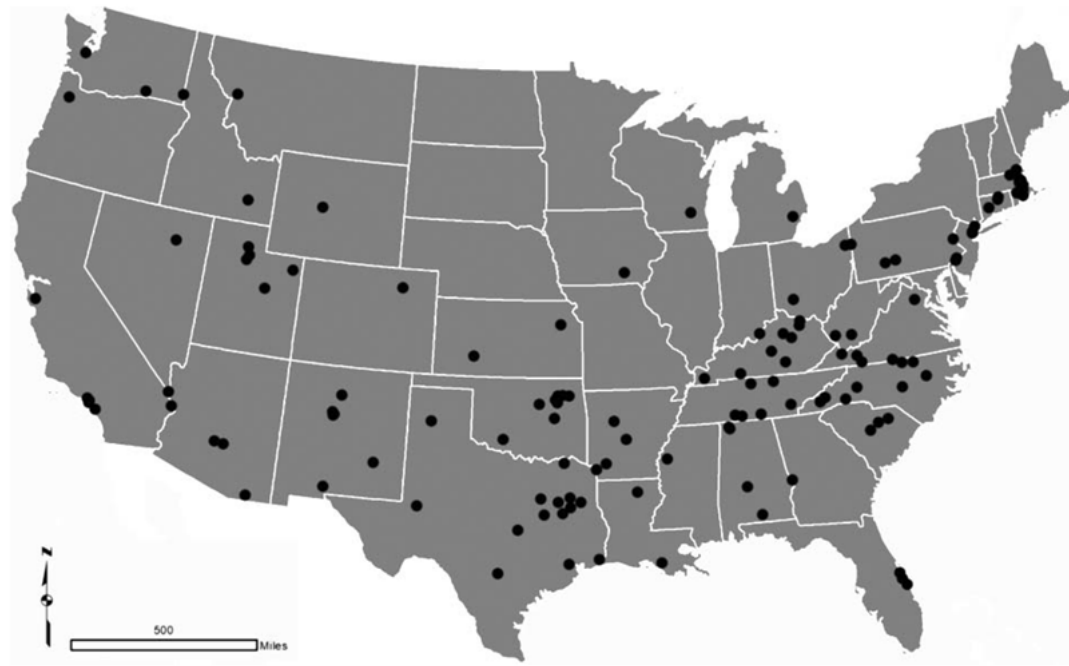
Steward Health Care, operator of 8 Massachusetts hospitals, files for bankruptcy



Private Equity Investment in Health Care Delivery

Hospital Acquisitions

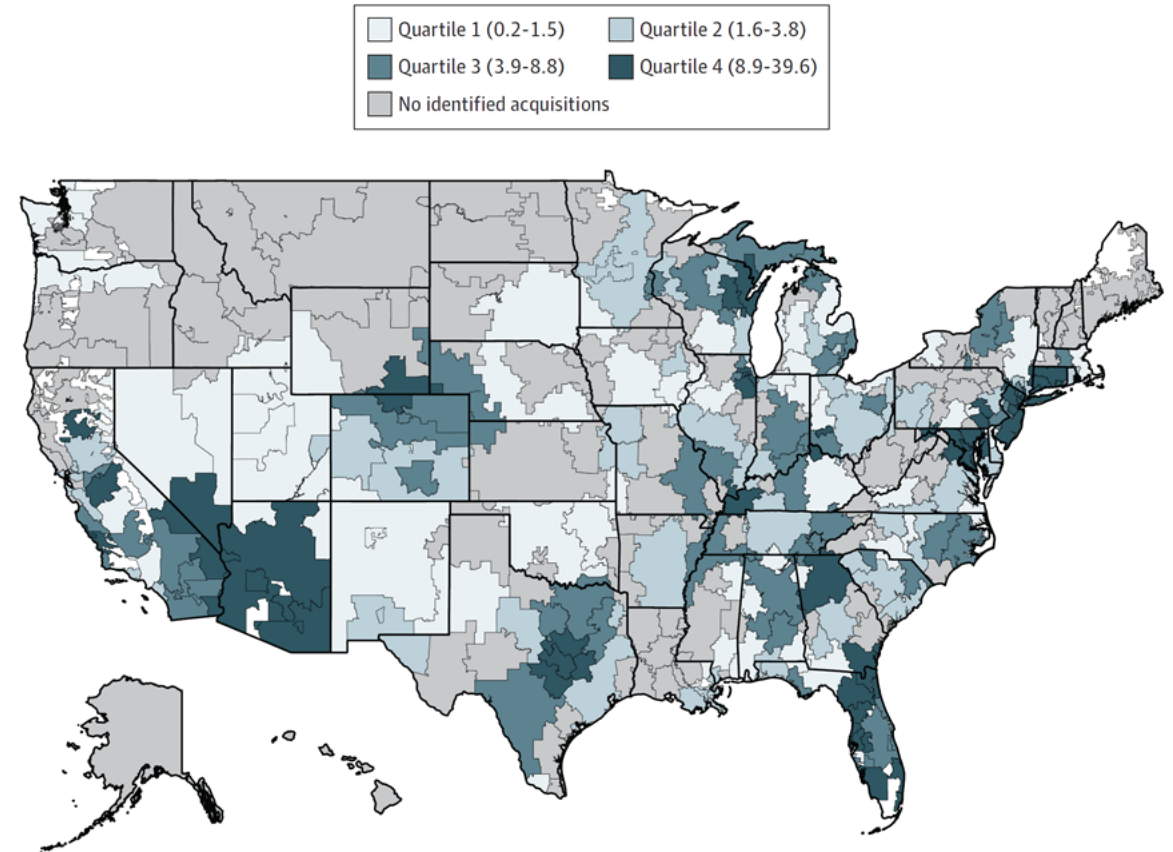
Figure. Locations of private equity-owned hospitals in 2018.



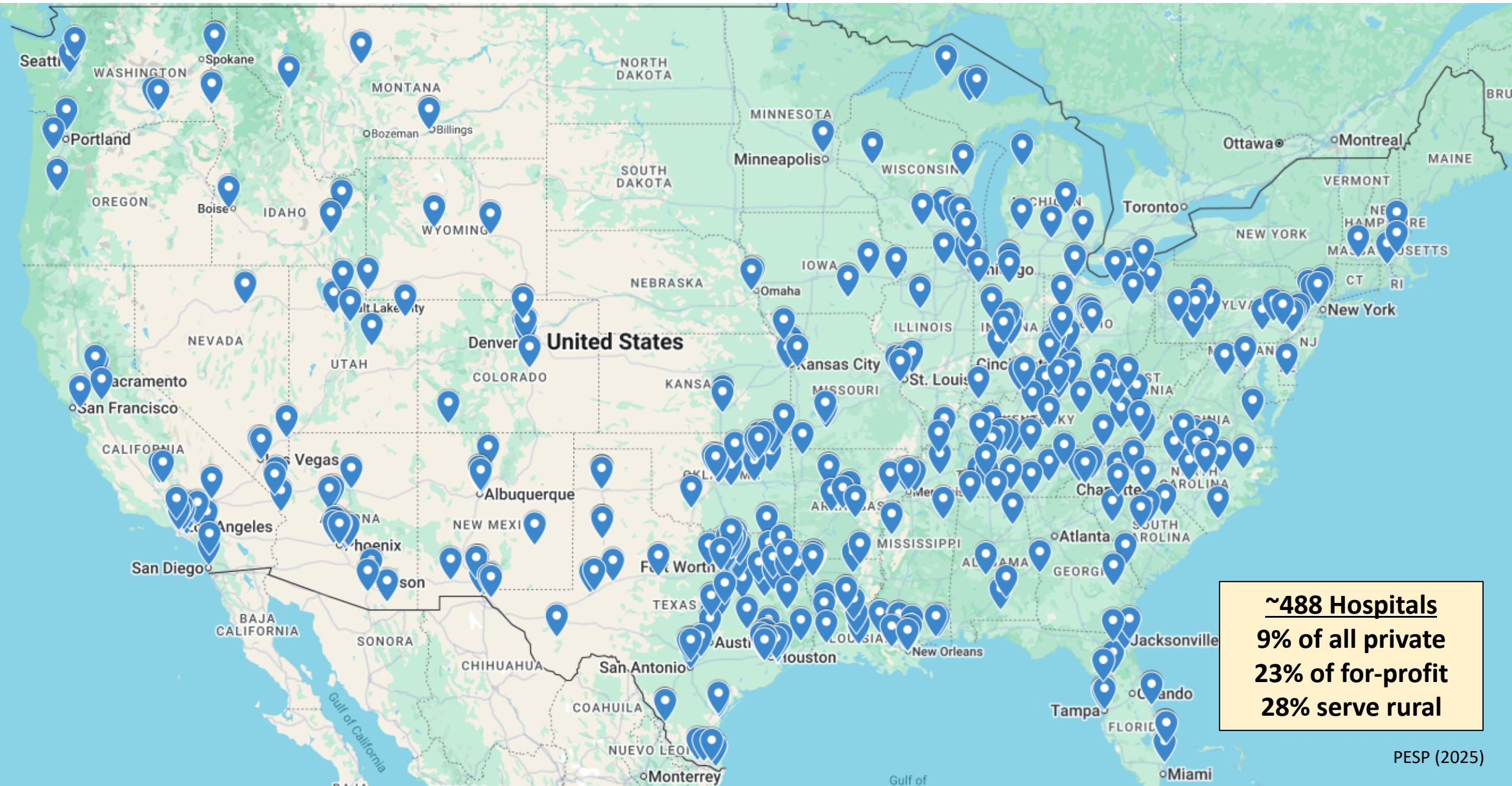
Using Medicare cost reports, the addresses for the 130 private equity-owned hospitals in 2018 were identified. There were no such hospitals located in Hawaii or Alaska.

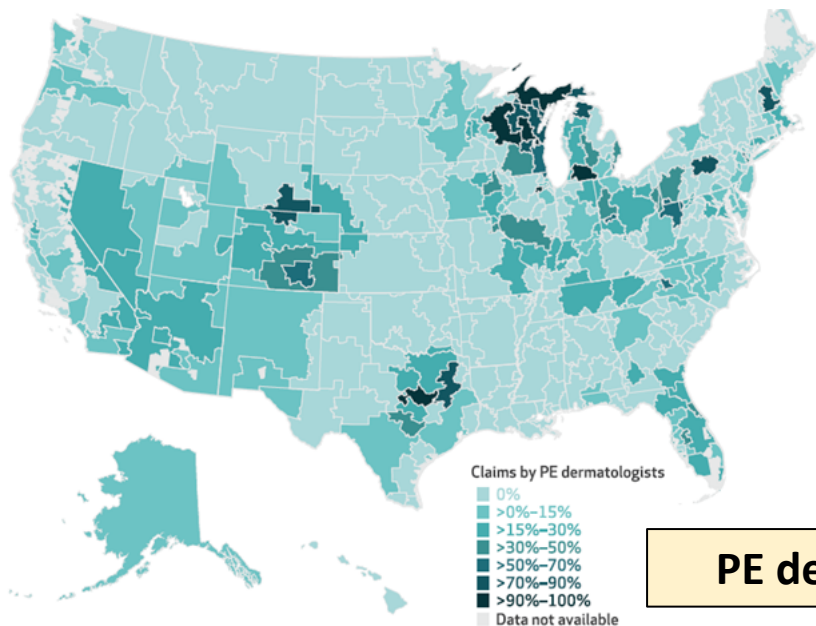
Physician Practice Acquisitions

Figure 1. Private Equity (PE) Penetration Across 6 Office-Based Specialties by Hospital Referral Region (HRR)



Private Equity Hospitals through 2025

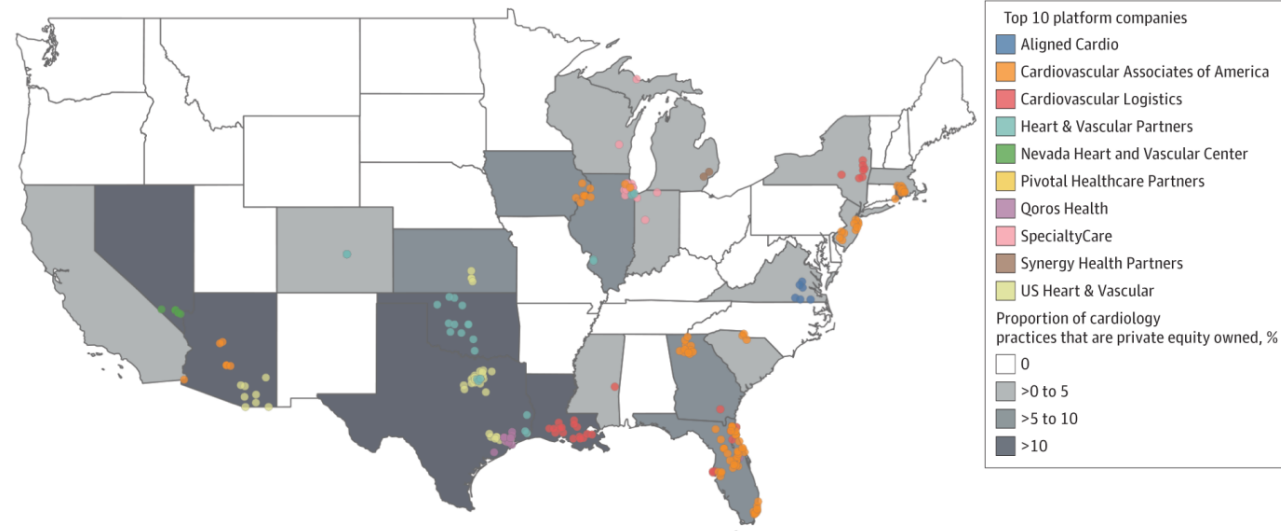




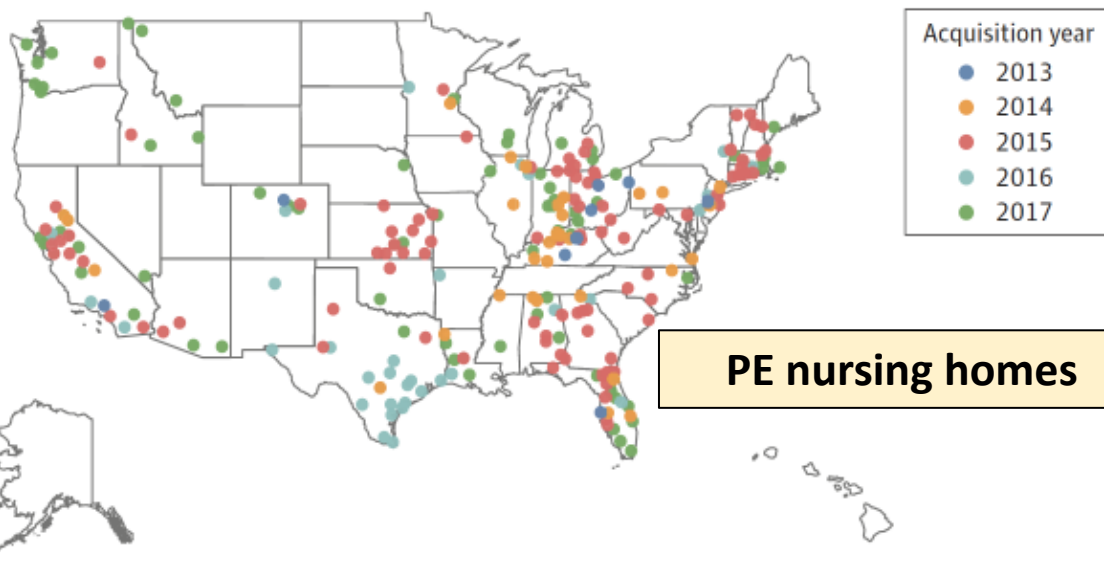
PE dermatology

Braun RT, Bond AM, Qian Y, Zhang M, Casalino LP. Health Affairs (2021)

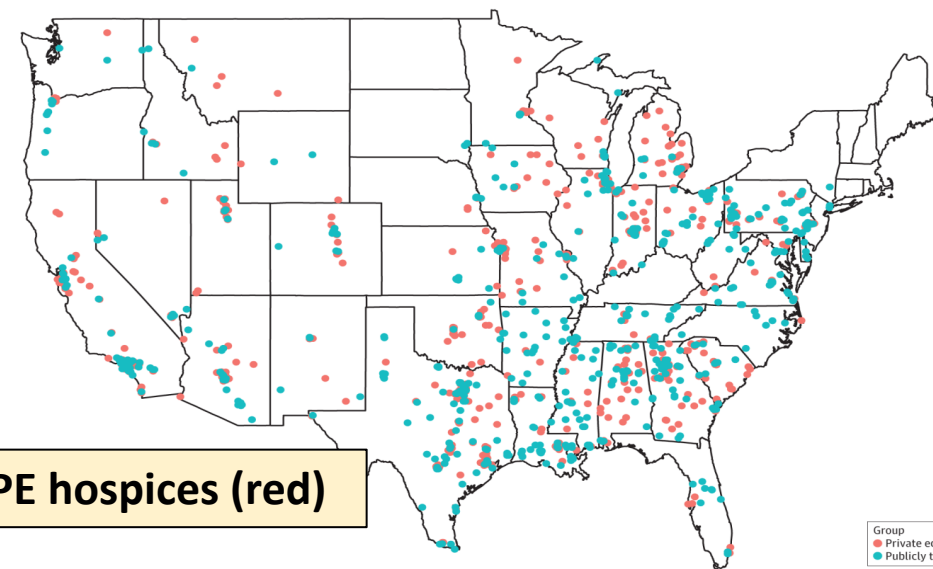
PE cardiology practices



Singh Y, Reddy M, Whaley C. JAMA Health Forum (2024)



PE nursing homes

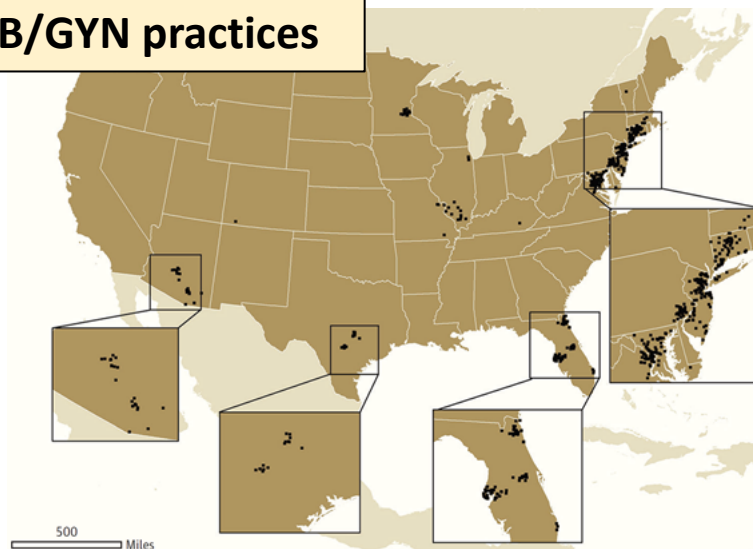


PE hospices (red)

Braun R, Jung HY, Casalino L, Myslinksi L, Unruh M. JAMA Health Forum (2021)

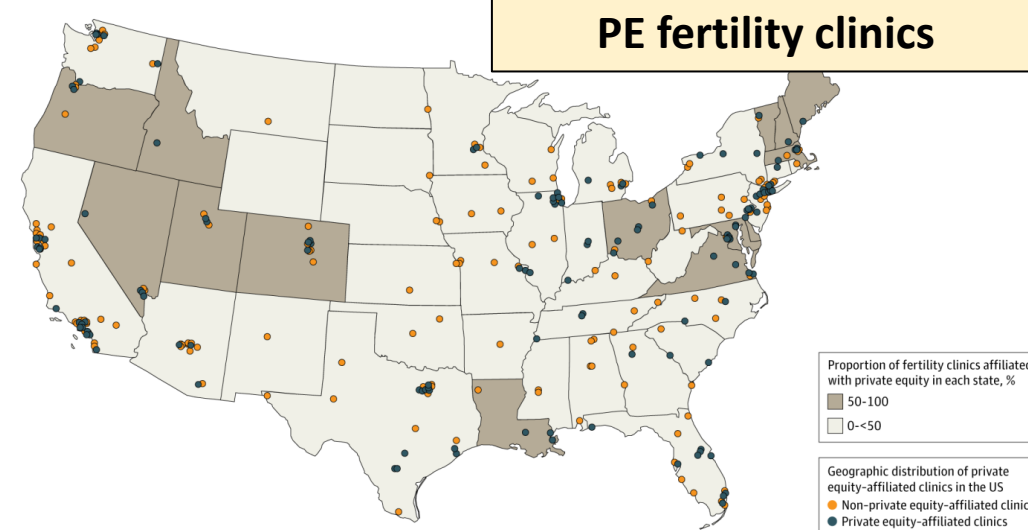
Braun RT, Unruh MA, Stevenson DG, et al. JAMA Network Open (2023)

PE OB/GYN practices

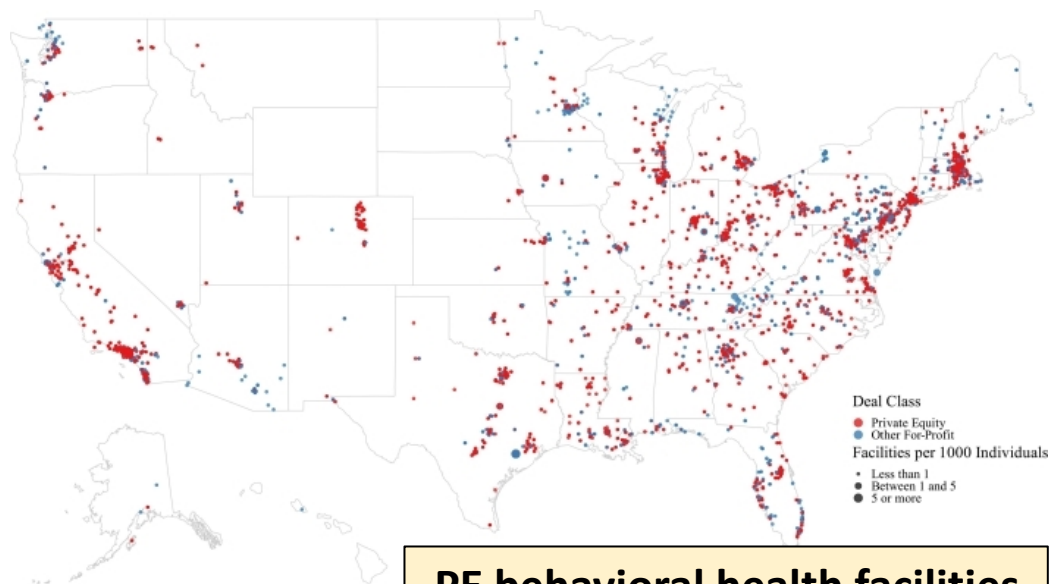


Bruch JD, Borsa A, Song Z, Richardson SS. JAMA Internal Medicine (2020)

PE fertility clinics

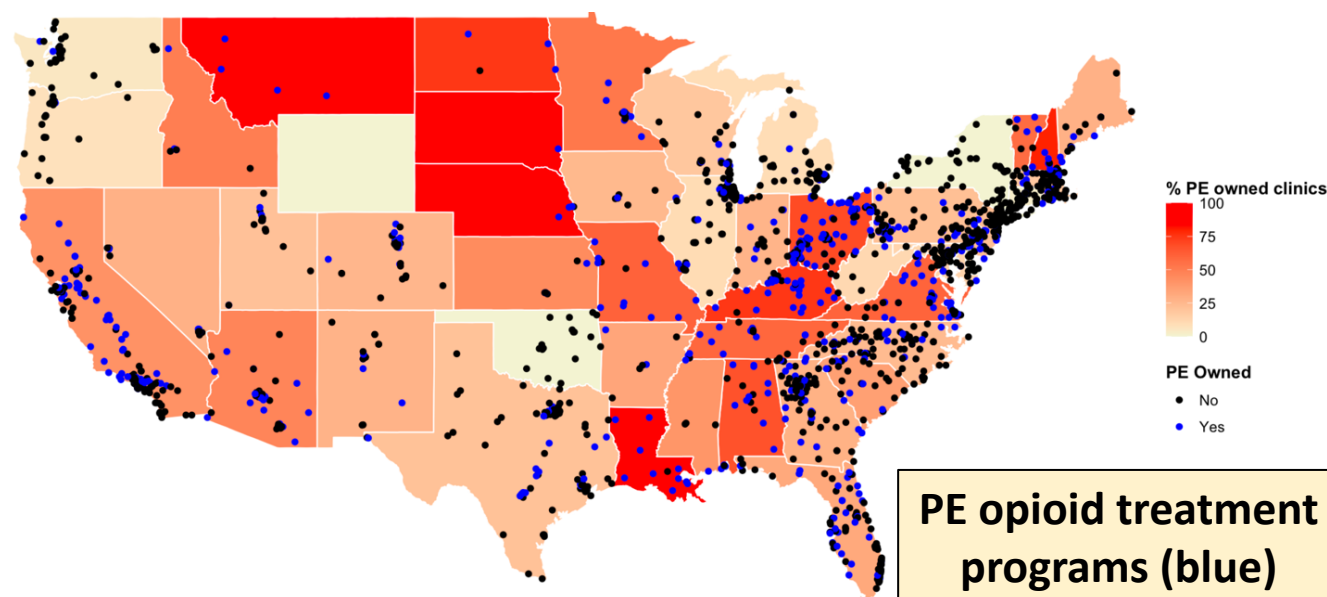


Ke J, Chen J, Chun E, Shahinian V, Dupree JM. JAMA (2025)



PE behavioral health facilities

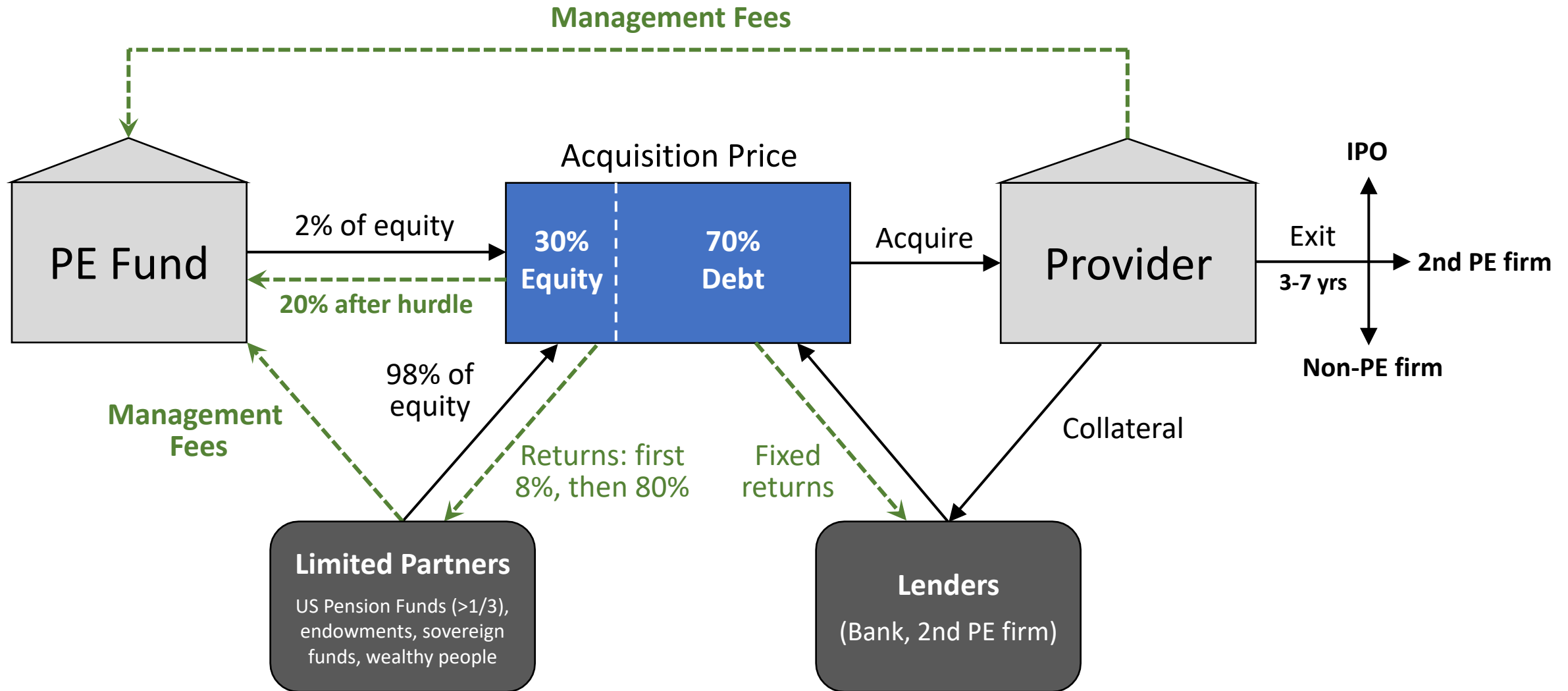
Thornburg B, McGinty EB, Eddebuettel J, et al. Health Affairs Scholar (2024)



PE opioid treatment programs (blue)

Zhu DT, Song Z, Kannan S, Cai C, Bajaj SS, Gondi S. JAMA Psychiatry (2025)

Classic Model of a Private Equity Leveraged Buyout (LBO)



Management fees = $\frac{2}{3}$ of revenue for PE general partner

Song Z. In progress

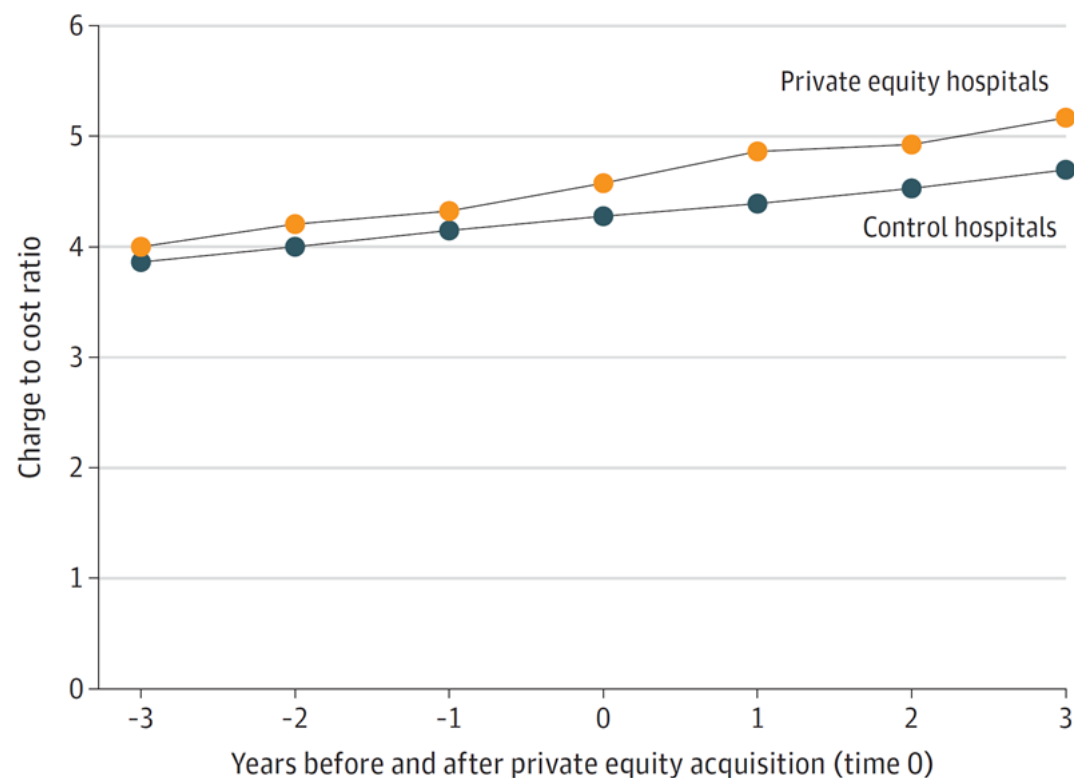
Changes in Hospital Income, Use, and Quality Associated With Private Equity Acquisition

Joseph D. Bruch, BA; Suhas Gondi, BA; Zirui Song, MD, PhD

Relative to control, PE acquisitions increased:

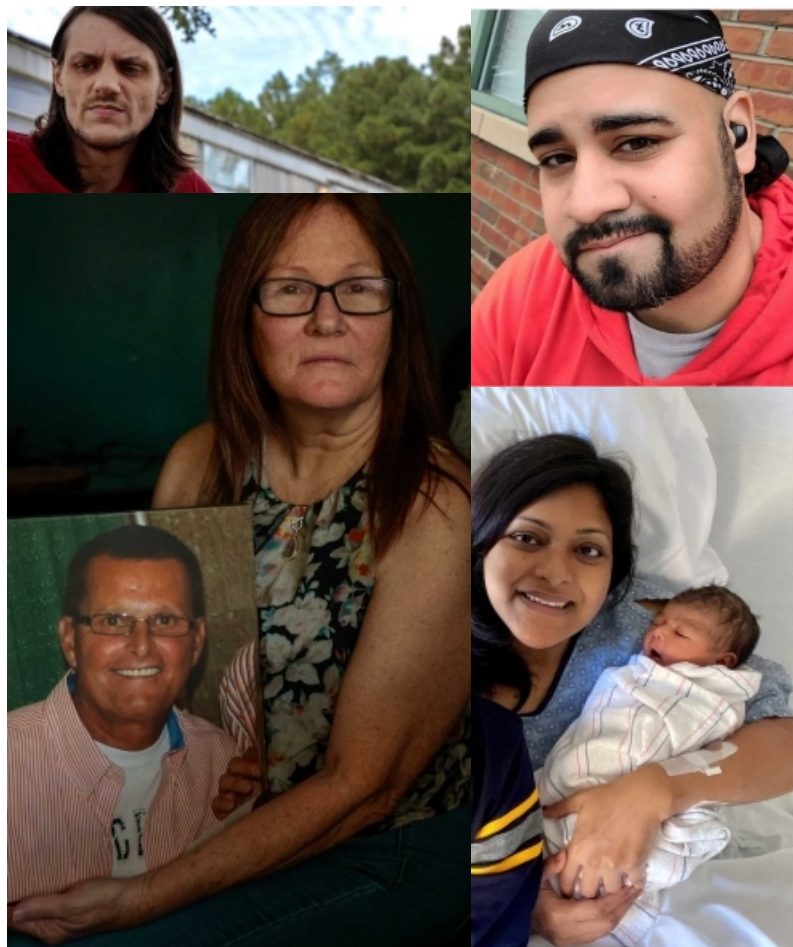
Net income	27%
Charges per day	7%
Charge/cost ratio	7%
Charge/cost ratio (ED)	16%
Case mix	1.4%
Medicare %	-2.4%

Figure. Total Charge to Cost Ratios Before and After Private Equity Acquisition



The Boston Globe

They died in hallways.
In line. Alone.



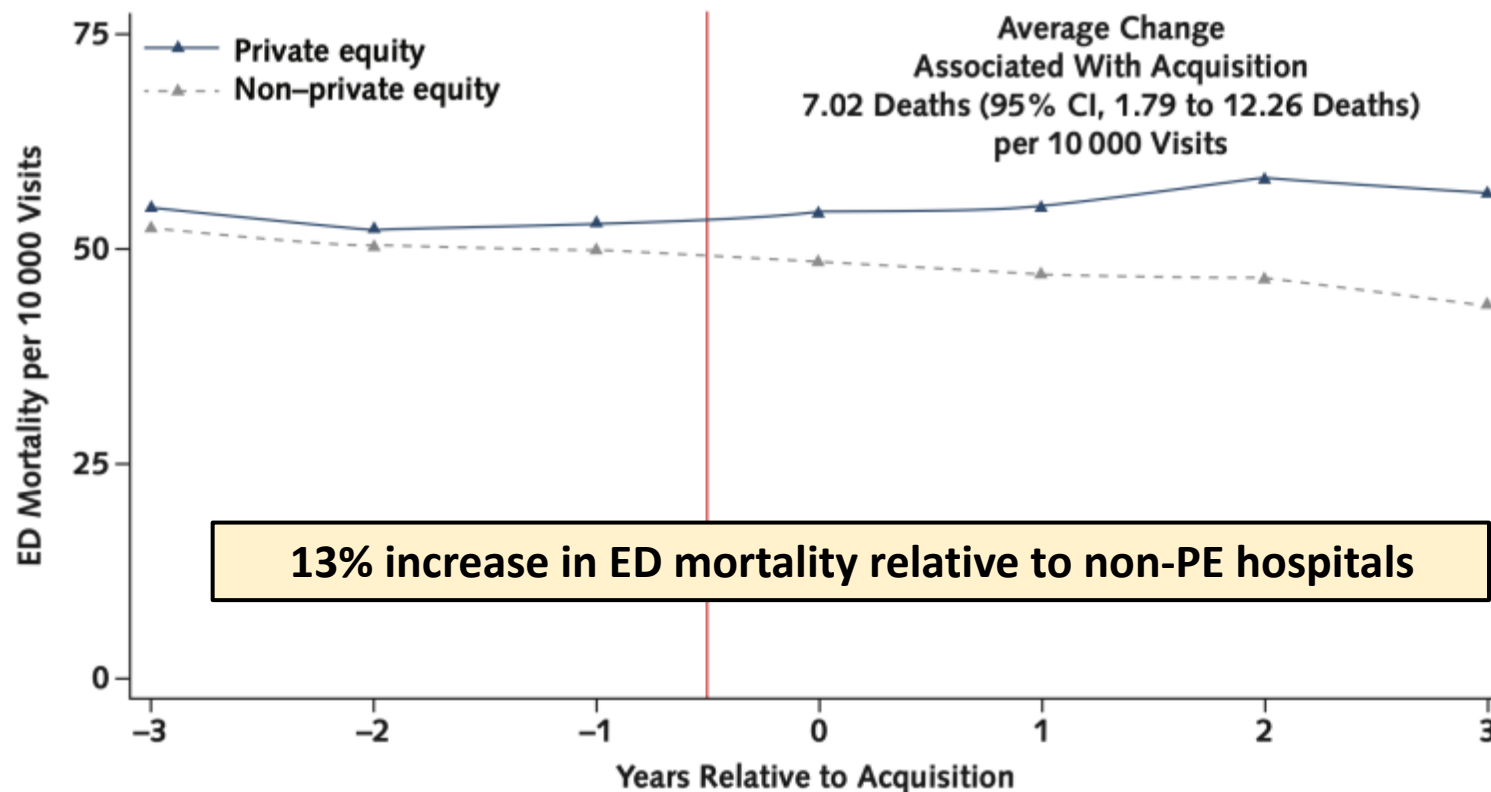
Annals of Internal Medicine (2025)

ORIGINAL RESEARCH

Hospital Staffing and Patient Outcomes After Private Equity Acquisition

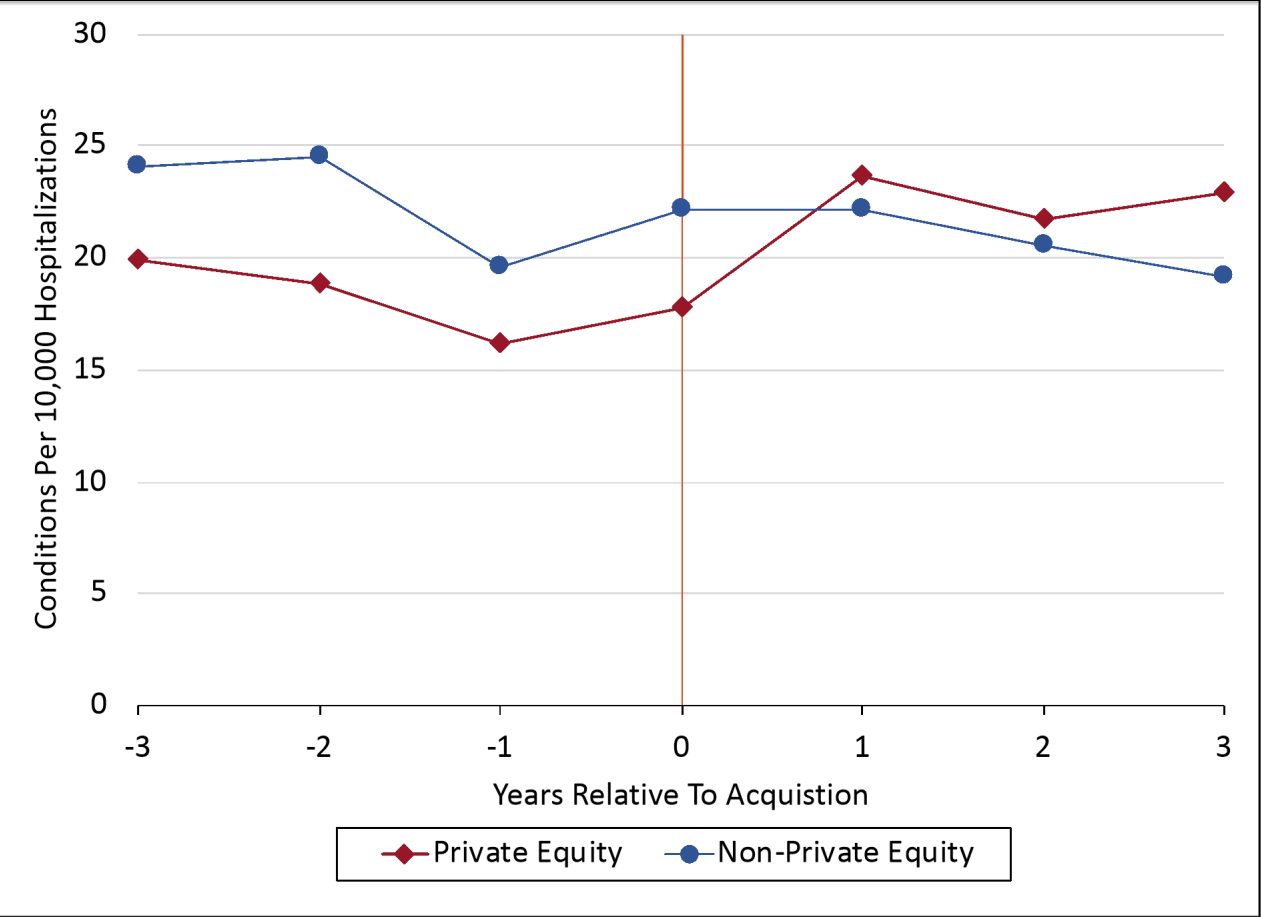
Sneha Kannan, MD, MS; Joseph Dov Bruch, PhD; José R. Zubizarreta, PhD; Jennifer Stevens, MD, MS; and Zirui Song, MD, PhD

Figure 3. Emergency department mortality at private equity and control hospitals.



Changes in Hospital Adverse Events and Patient Outcomes Associated With Private Equity Acquisition

Sneha Kannan, MD; Joseph Dov Bruch, PhD; Zirui Song, MD, PhD



Relative to control, PE acquisitions increased:

Composite HACs	25%
Falls	27%
Central line infections	38%
(Despite 16% fewer central lines)	

Surgical site infections doubled at PE hospitals, while declining at controls. (Despite 8% fewer surgeries performed)

... among younger and lower-risk Medicare patients (↓ duals)

... and after a 12% ↑ in transfers



Changes in hospital financial performance and quality of care after real estate investment trust acquisition: quasi-experimental difference-in-differences study

87 REIT acquired hospitals vs.
337 non-REIT acquired matched controls

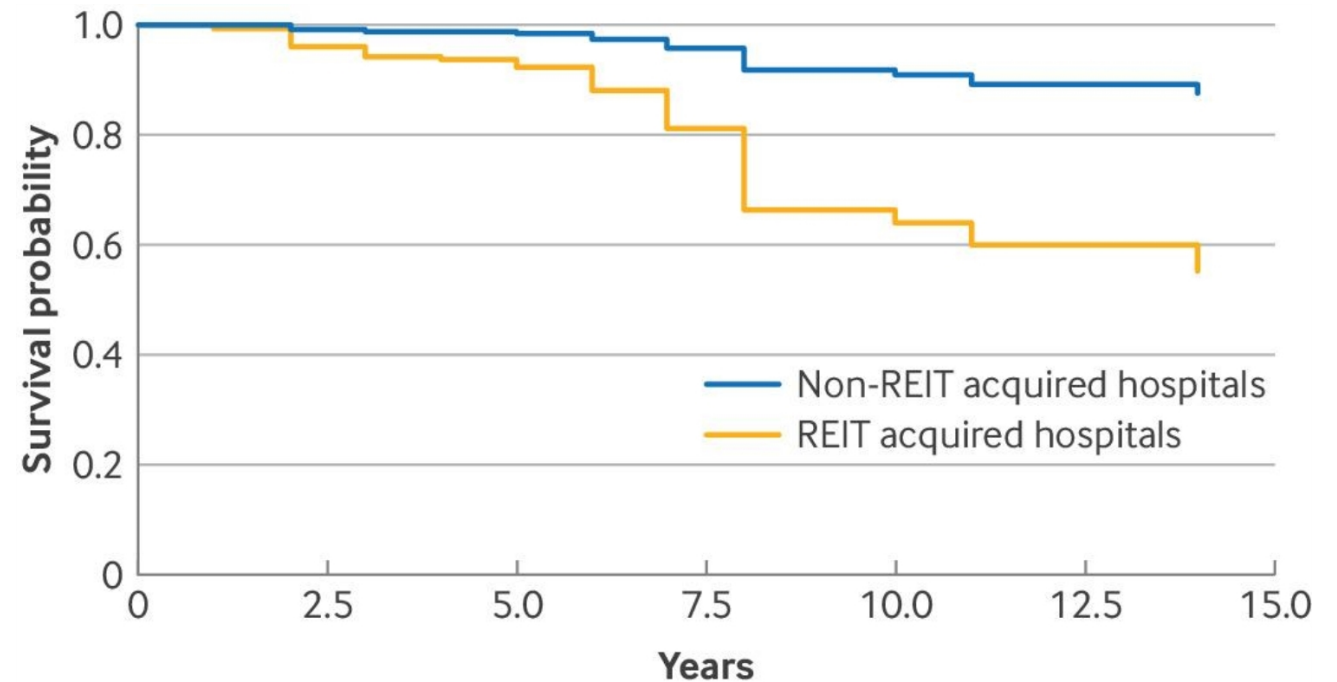
WHAT IS ALREADY KNOWN ON THIS TOPIC

Real estate investment trusts (REITs) have emerged as a growing financial force in the delivery of healthcare
Empirical data on the impacts of REITs on hospital finances and quality of care are limited

WHAT THIS STUDY ADDS

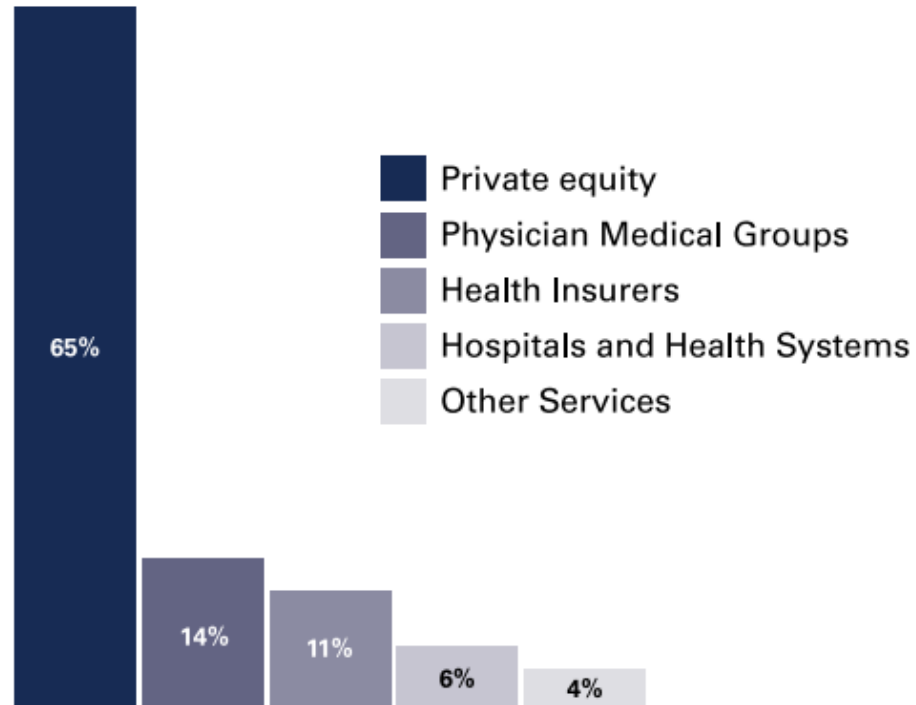
This study found that REIT acquisitions were not associated with changes in most financial or quality of care measures
REIT acquired hospitals were, however, at greater risk of closure and bankruptcy
These findings highlight the need for regulatory oversight of REIT acquisitions of hospitals

“25% of REIT acquired hospitals closed or filed for bankruptcy compared with 4% of controls.”



Private Equity and Physician Practices

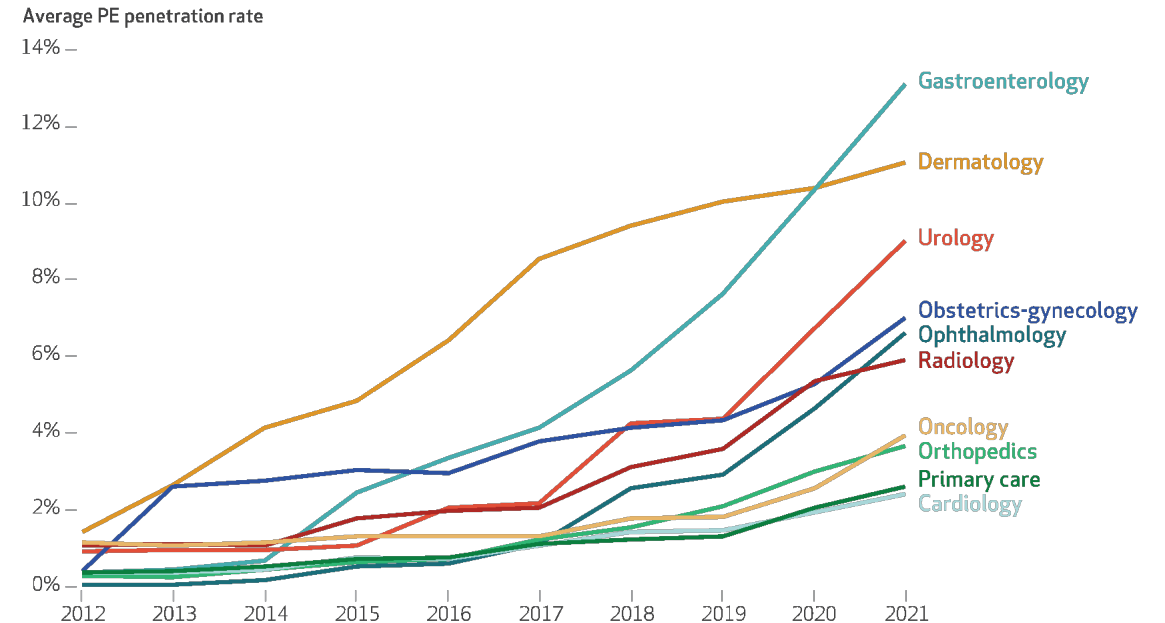
Percentage of acquired physicians by type, 2019 - 2023



LevinPro HG, Levin Associates, 2023, June, levinassociates.com. Only includes values for deals reported. Certain acquirer types were also modified to more closely align with the services provided.

EXHIBIT 2

Trends in private equity (PE) penetration at the physician level in the US among 10 physician specialties, 2012-21



SOURCE Authors' analysis of data from the Irving Levin Associates Healthcare M&A Database, PitchBook private equity and merger and acquisition database, and OneKey Database provided by IQVIA (2020-21) and SK&A Office Based Physicians Database provided by IMS Health (now IQVIA) (2012-19). The PitchBook data presented here have not been reviewed by PitchBook analysts. The PitchBook database is dynamic; data for this figure are as of June 15, 2022. **NOTE** Average PE penetration rates at the physician level in each year by specialty were calculated by weighting each Metropolitan Statistical Area (MSA)-level market share by the number of full-time-equivalent physicians in that MSA by specialty, equivalent to the US penetration rate.

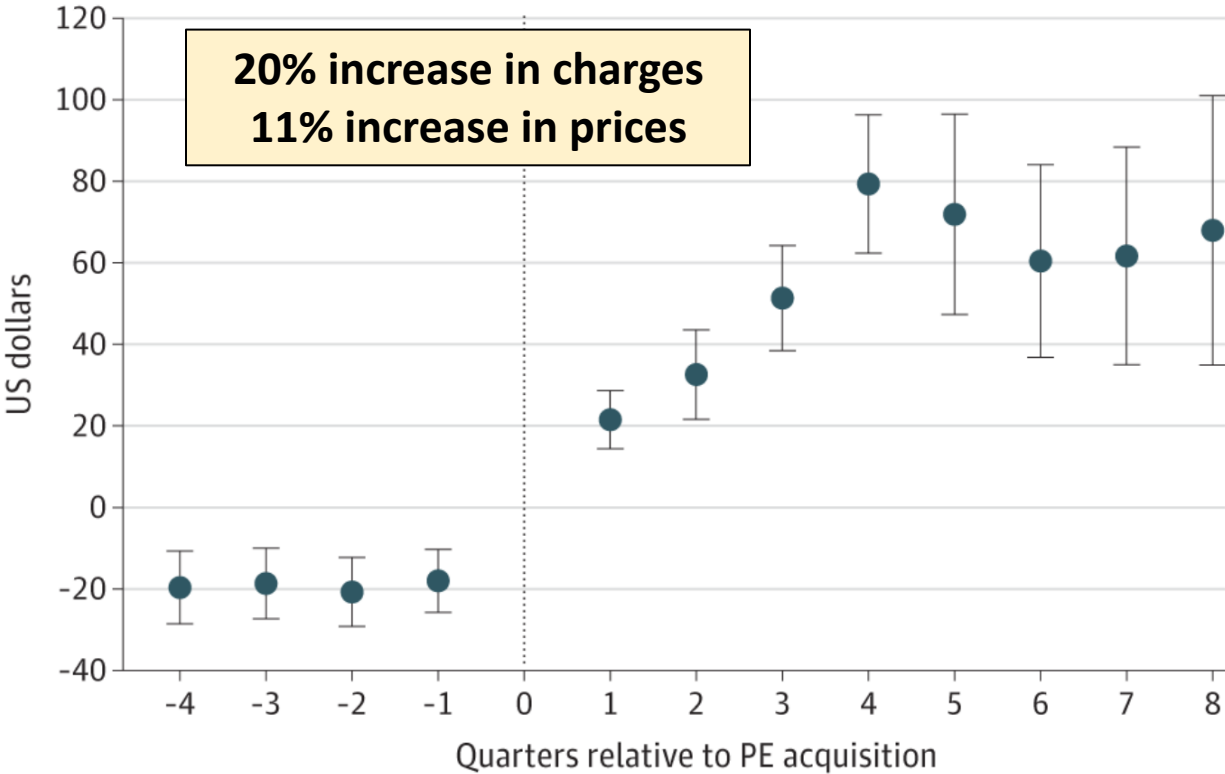
From 816 practices in 2012 to 5,779 practices in 2021 across 307 Metropolitan Statistical Areas (MSAs)
1/3 of MSAs: single PE firm has >30% market share

Association of Private Equity Acquisition of Physician Practices With Changes in Health Care Spending and Utilization

Yashaswini Singh, MPA; Zirui Song, MD, PhD; Daniel Polsky, PhD, MPP; Joseph D. Bruch, PhD; Jane M. Zhu, MD, MPP, MSHP

Table 1. Characteristics of PE- and Non-PE-Acquired Physician Practices at Baseline, 2015

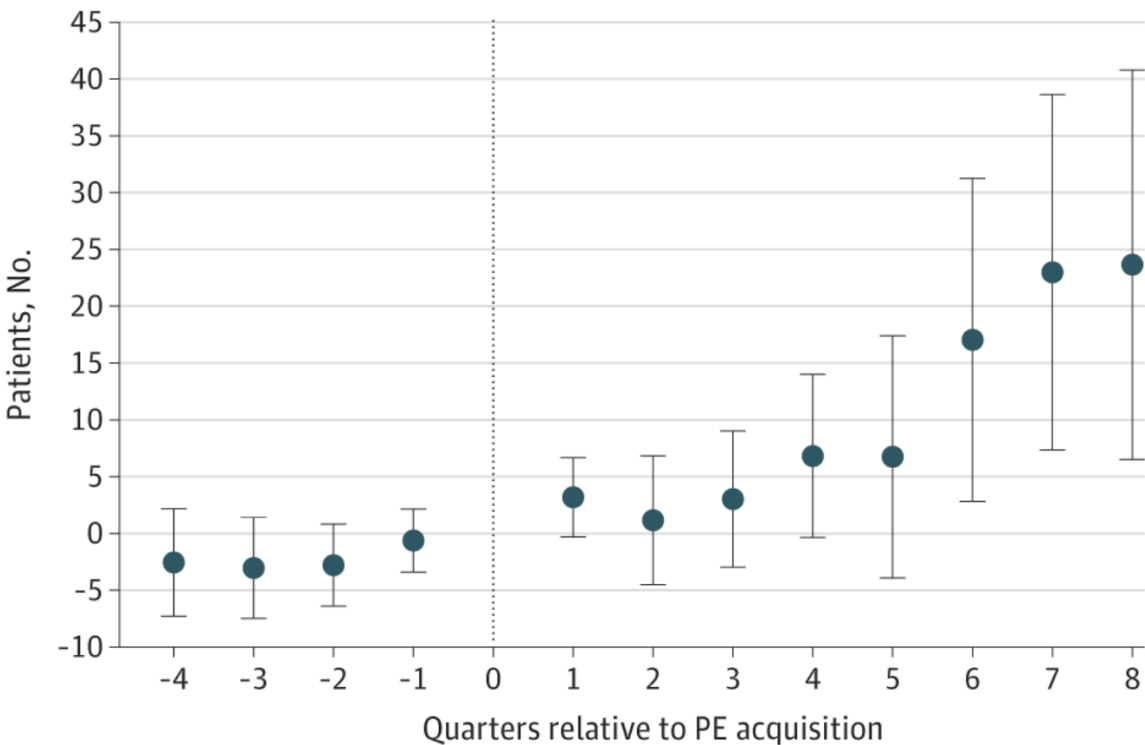
Characteristic	Mean (SD)	
	PE-acquired	Non-PE-acquired ^a
Physician practices, No.	578	2874
Charge/claim, mean \$	322 (258)	332 (326)
Allowed amount/claim, mean \$	187 (136)	178 (136)
Total No.		
Unique patients	94 (182)	88 (172)
New patients	72 (136)	67 (132)
Encounters	124 (237)	118 (224)
E&M visits	75 (188)	72 (180)
Share of E&M visits >30 min		
New patients	0.26 (0.15)	0.26 (0.21)
Established patients	0.19 (0.17)	0.18 (0.22)
Patient HCC score, median	1.21 (1.05)	1.28 (1.10)



Association of Private Equity Acquisition of Physician Practices With Changes in Health Care Spending and Utilization

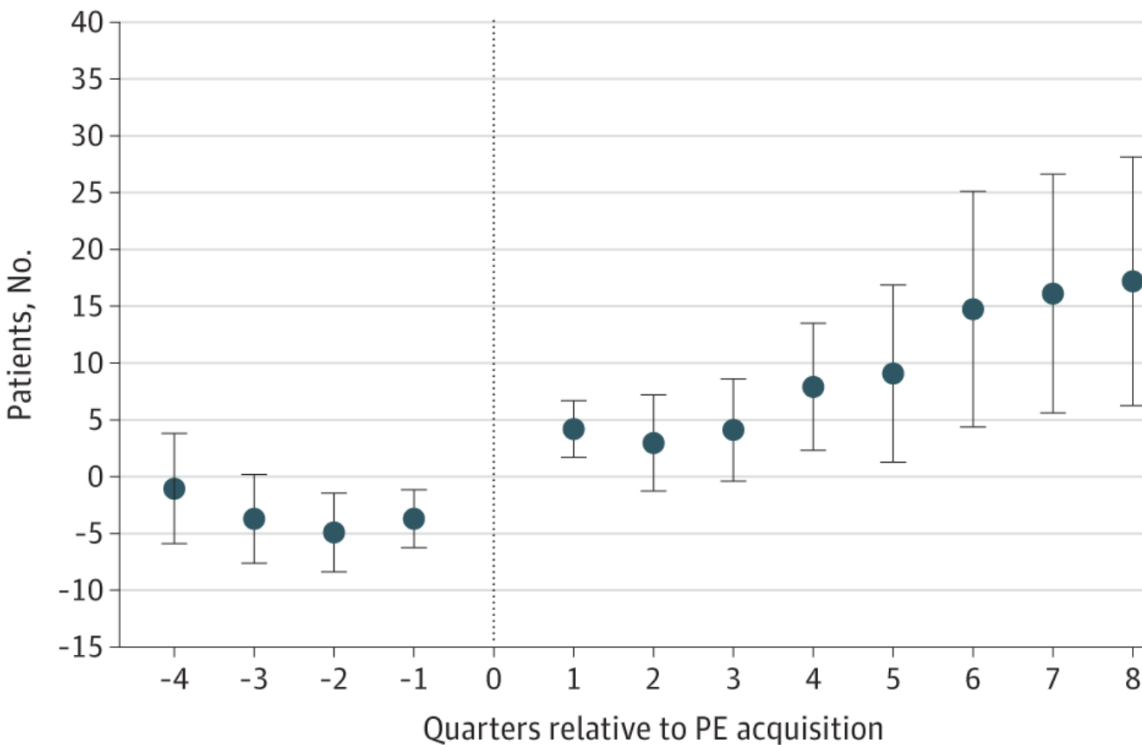
Yashaswini Singh, MPA; Zirui Song, MD, PhD; Daniel Polsky, PhD, MPP; Joseph D. Bruch, PhD; Jane M. Zhu, MD, MPP, MSHP

A Unique patients per practice



26% increase in unique patients
38% increase in new patient visits
9% increase in long (>30 min) visits

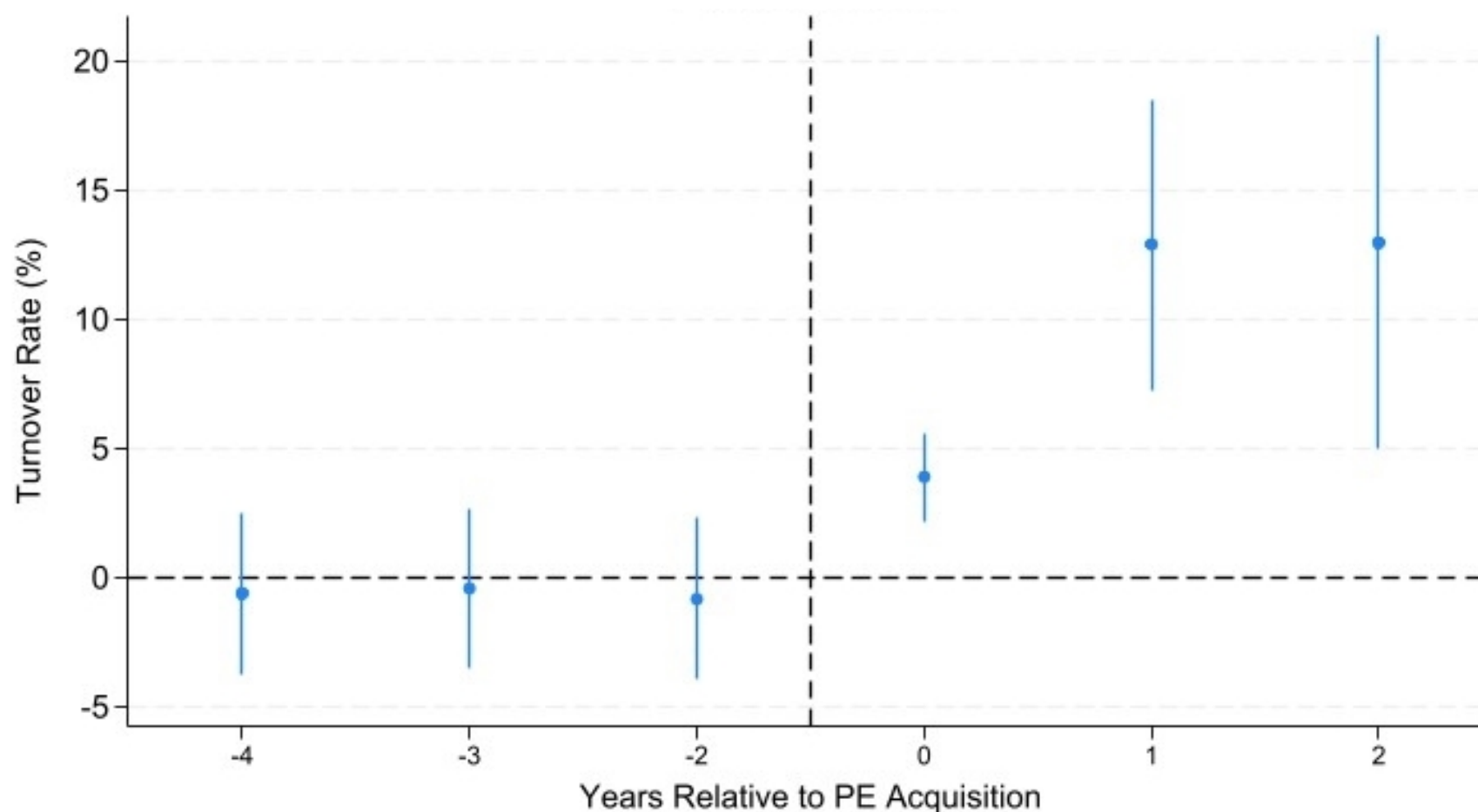
B New patients per practice



Workforce Composition In Private Equity-Acquired Versus Non-Private Equity-Acquired Physician Practices

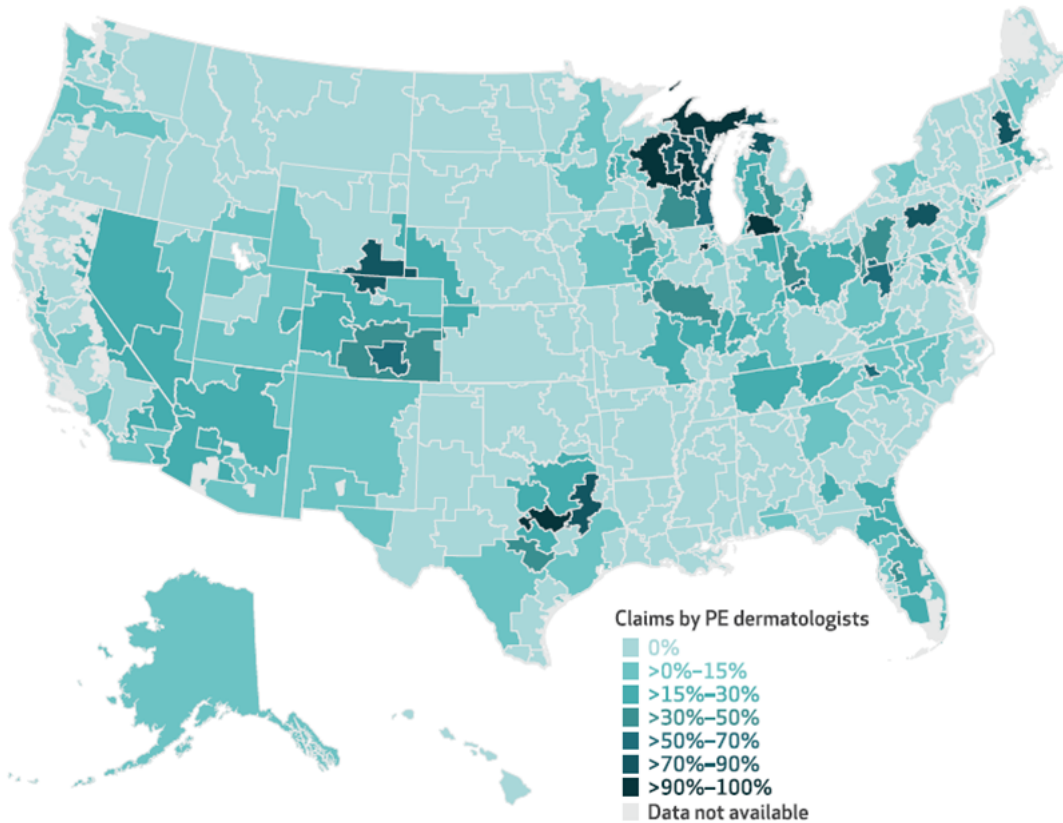
Relative to control, PE increases APP hiring.

PE acquisitions raised physician turnover rate by 13 percentage points (265% over baseline)

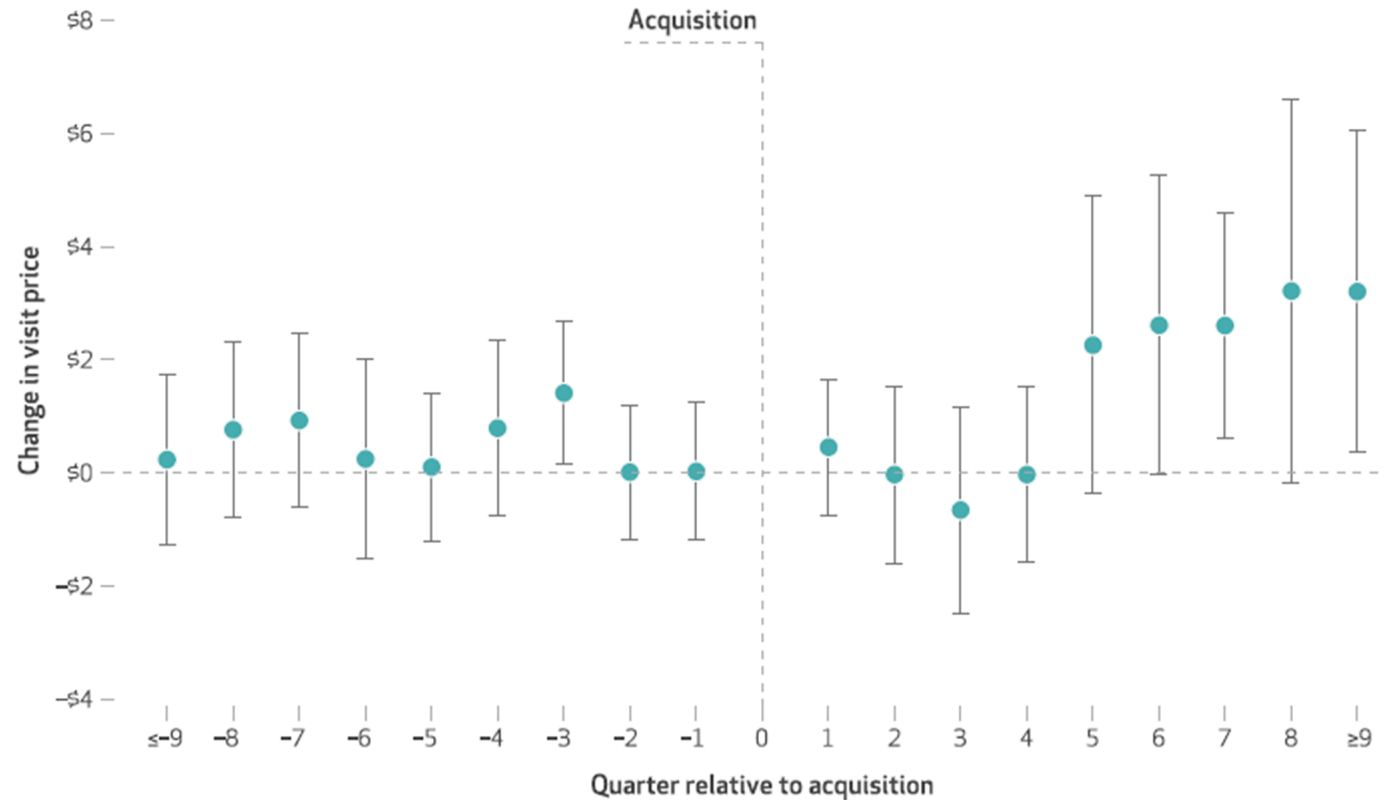


Additional Evidence on Physician Practice Acquisitions

Percent of dermatologist claims made by private equity (PE) dermatologists, by hospital referral region, 2017



Effect of private equity acquisition on the price of a routine dermatology office visit, by quarter, 2012–17



“At 1.5 years after acquisition, prices paid to private equity dermatologists for routine medical visits were 3-5 percent higher than those paid to non-private equity dermatologists. There was no significant consistent impact on dermatology spending or use of biopsies, lesion destruction, or Mohs surgery.”

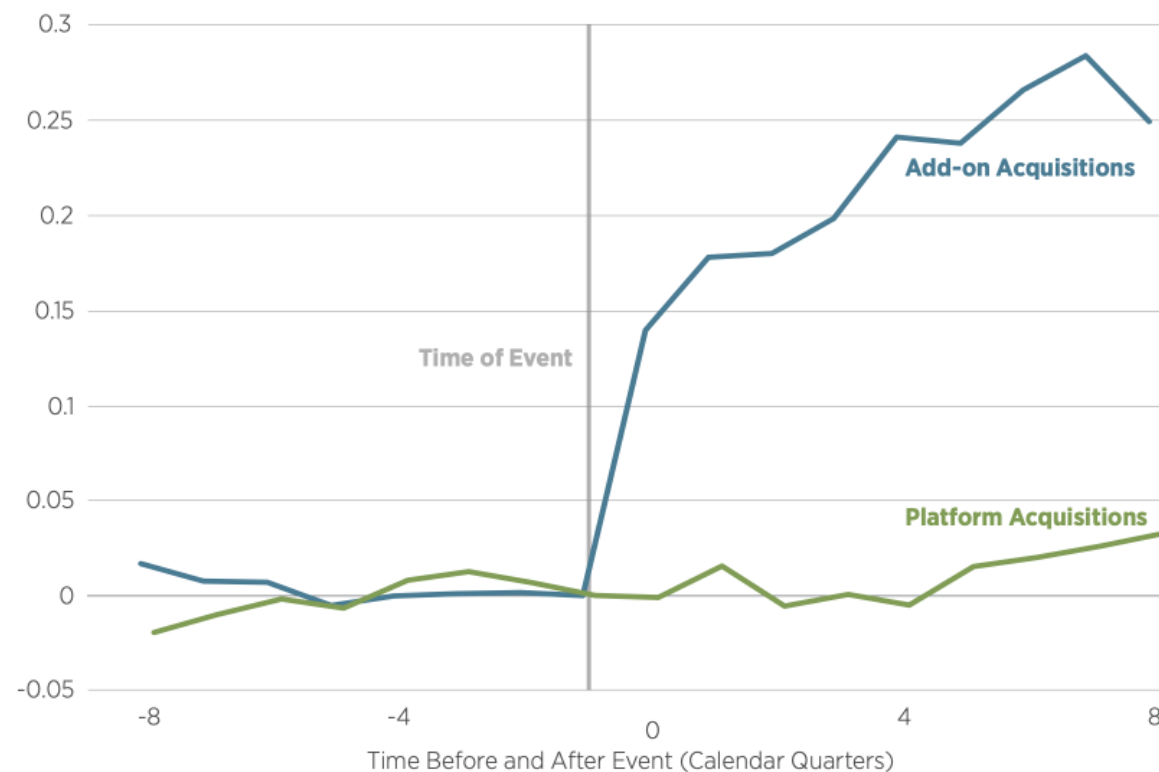
Painful Bargaining: Evidence from Anesthesia Rollups

Based on BFI Working Paper No. 2024-149, "Painful Bargaining: Evidence from Anesthesia Rollups," by Aslihan Asil, Yale University; Paulo Ramos, University of Chicago; Amanda Starc, Northwestern University; and Thomas G. Wollmann, University of Chicago

Rollups in the anesthesia industry drive consolidation and significant price increases, particularly in already concentrated markets. Court-ordered divestitures and other remedies show potential for reducing these anti-competitive effects.

"Serial acquisitions of clinician practices backed by investment funds have consolidated a geographically dispersed set of markets, consistent with the... rollup as a "buy-and-build" strategy. **Prices increased by approximately 35% following add-on acquisitions.** ... the increases reflect the internalization of business-stealing effects—**less competition**—rather than other explanations, such as changes in quality, negotiating ability, or pricing tactics employed by sponsors."

Figure 1 • Prices Rise Following Add-On but not Platform Acquisitions



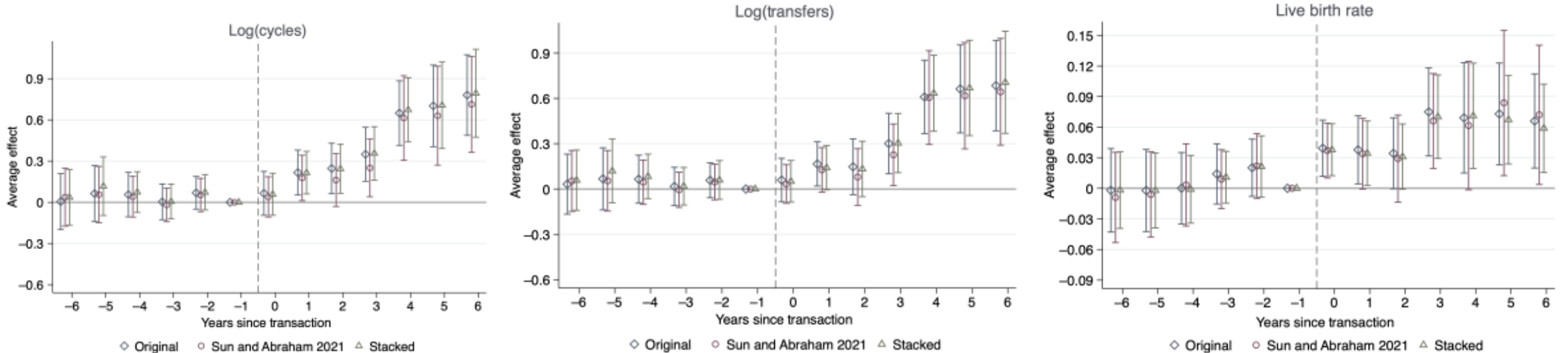
Note: This figure plots the average anesthesia price on the vertical axis against time on the horizontal axis. The vertical gray line corresponds to the quarter of the acquisition. Prices are normalized to zero in that quarter. The blue line corresponds to add-on acquisitions; the green line corresponds to platform acquisitions (i.e., the first acquisition by a sponsor in a market).

Getting Down to Business: Chain Ownership and Fertility Clinic Performance

MANAGEMENT SCIENCE
Vol. 71, No. 6, June 2025, pp. 5022–5044
ISSN 0025-1909 (print), ISSN 1526-5501 (online)

Ambar La Forgia,^{a,*} Julia Bodner^b

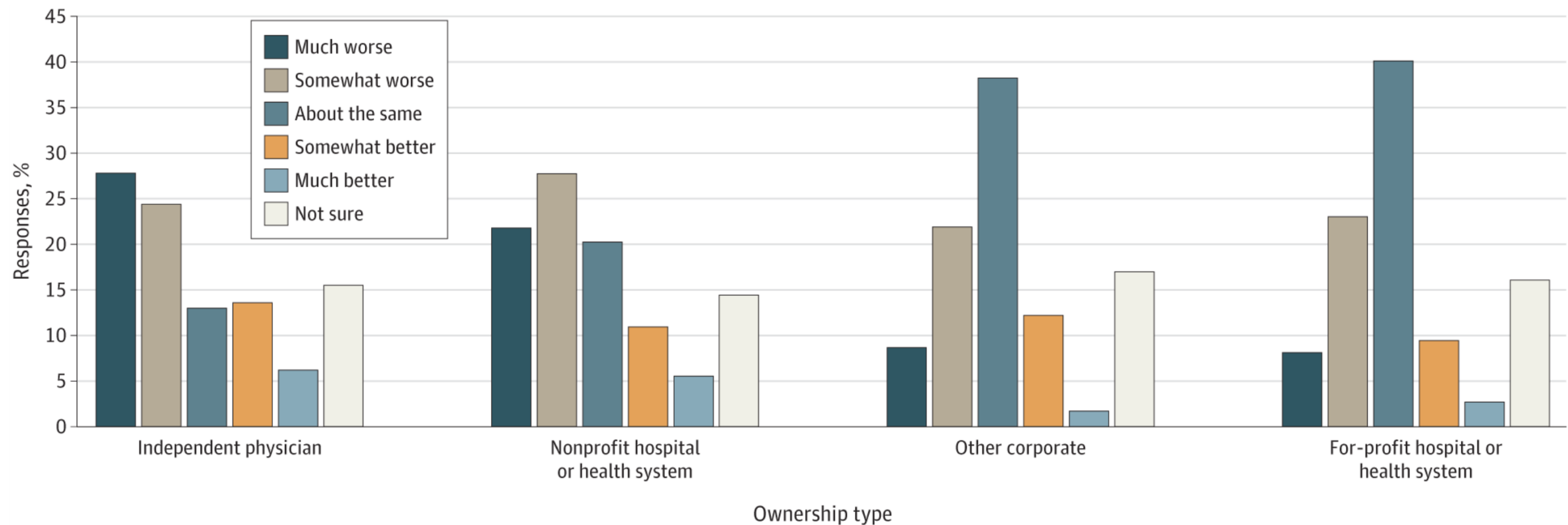
“IVF cycles increase by 27.2%, and IVF success rates increase by 13.6% after acquisition by a fertility chain.”



“We find almost all the **volume effect for acquired clinics occurs because of PE funding** (13.3% increase in cycles without PE and 30.4% increase with PE). Importantly, PE funding does not influence the live birth rate: live birth rates increase by 5.8 percentage points without PE funding and 5.0 percentage points with PE funding. **This result suggests quality improvements occur because of the chain, not the PE funding, whereas clinic growth is more reliant on PE funding.**”

Physician Perceptions of Private Equity

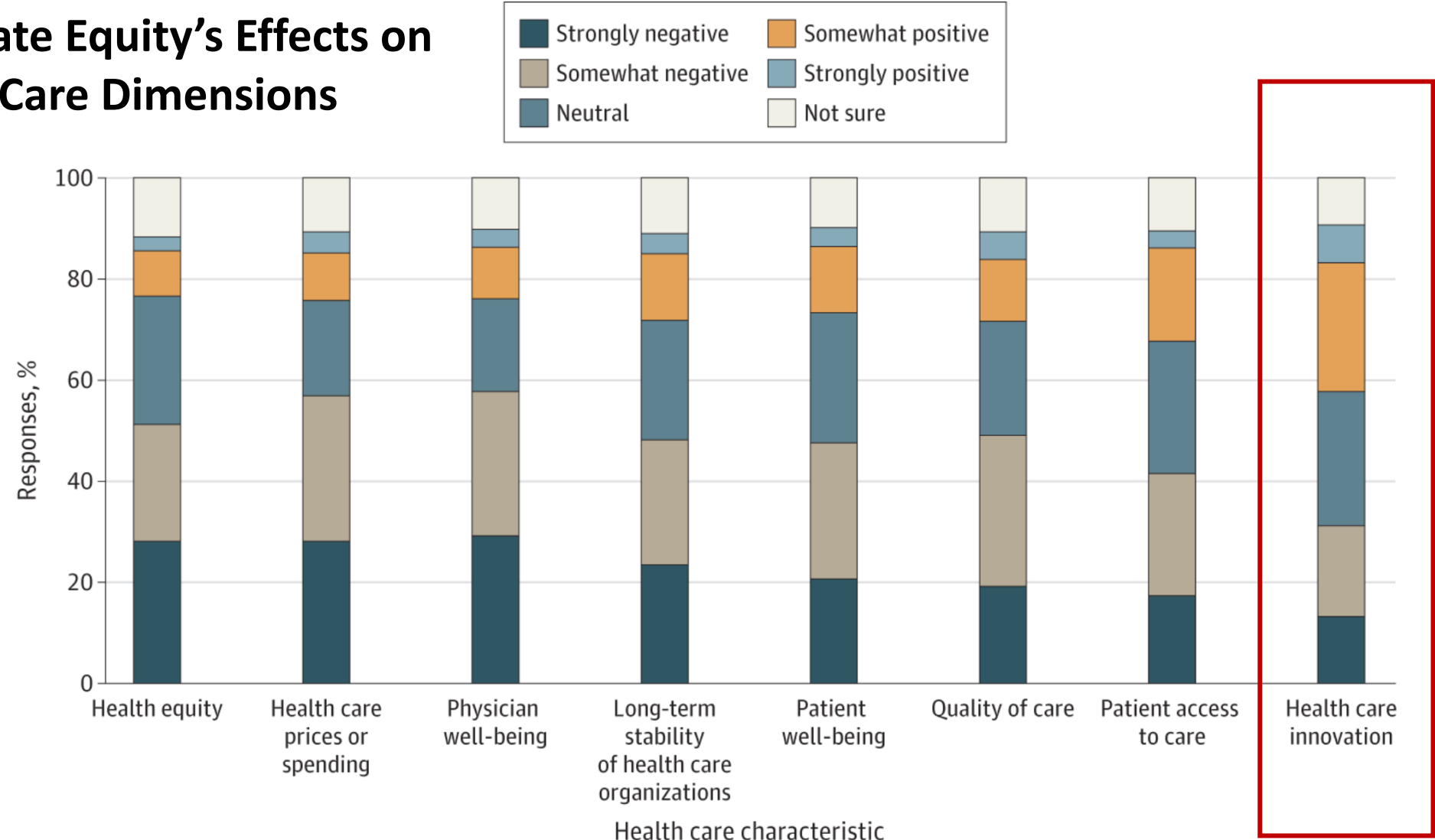
Favorability of Private Equity Ownership Compared With Other Types of Ownership



N=1397 MDs

Physician Perceptions of Private Equity

Perspectives of Private Equity’s Effects on Various Health Care Dimensions



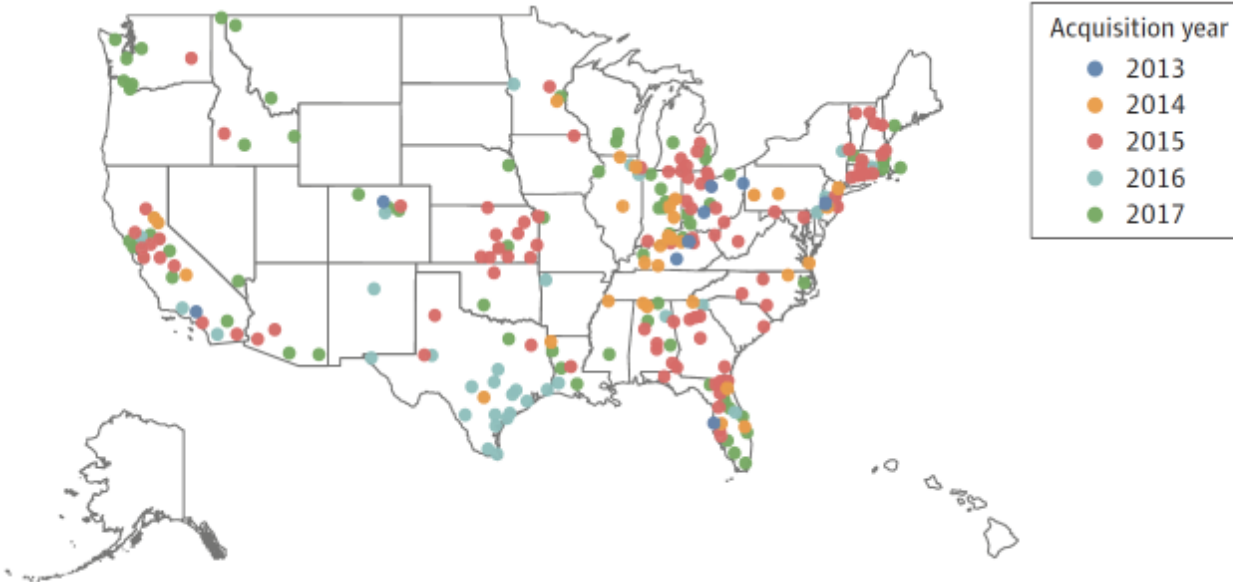
N=1397 MDs

Private Equity Acquisitions of Nursing Homes

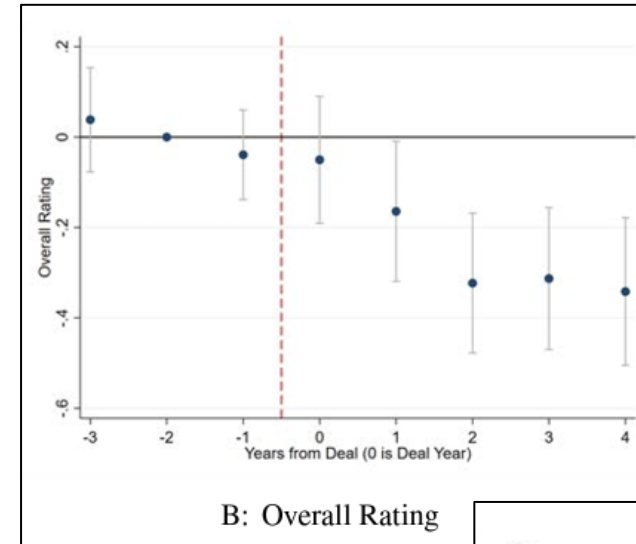
Original Investigation

Association of Private Equity Investment in US Nursing Homes With the Quality and Cost of Care for Long-Stay Residents

Robert Tyler Braun, PhD; Hye-Young Jung, PhD; Lawrence P. Casalino, MD, PhD; Zachary Myslinski, MD; Mark Aaron Unruh, PhD

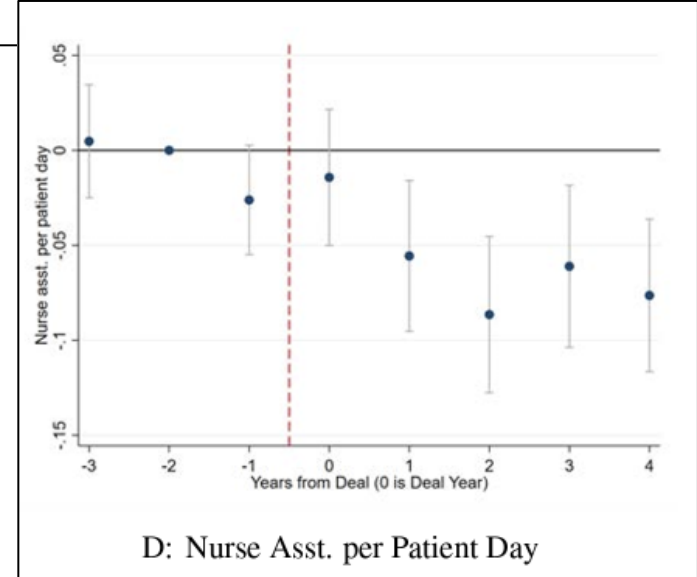


“...private equity acquisition of nursing homes was associated with **higher costs and increases in ED visits and hospitalizations for ambulatory sensitive conditions.**”



“PE ownership increases mortality by 11%.

Declines in measures of patient well-being, nurse staffing, and compliance with care standards help to explain the mortality effect.”



The
Review of
Financial
Studies

A Policy Framework for the Growing Influence of Private Equity in Health Care Delivery

F	Fraud & abuse	Enforcement of federal statutes: e.g. Anti-Kickback, Stark Laws
A	Antitrust	A) Federal: staffing and oversight at the Federal Trade Commission B) States: Attorneys General, "Corporate Practice of Medicine" laws
M	Moral hazard	A) Strengthen the accountability of PE firms to their acquired entities B) Address the % debt placed on the acquired entity (financial risk) C) Address the 20% tax rate for profits ("carried interest")
P	Patients & prices	A) Protect patients from out-of-network (surprise) balance bills B) Address high prices to protect employers, states, and taxpayers
T	Transparency	A) Consider the threshold (\$134 million) for reporting of acquisitions B) Public reporting of owners and investors (MA, IN, other states)

JAMA Forum

The New Role of Private Investment in Health Care Delivery

David M. Cutler, PhD; Zirui Song, MD, PhD

(2024)

Disruptive innovation as a business philosophy has brought benefits to many parts of the economy. But in health care delivery, evidence increasingly suggests that not all disruption creates value for patients. A central issue for policy is how to encourage truly value-adding innovation for patients and payers without hurting patients or bankrupting society.